Adult treatment scope · NCCP 00702a / 00702b This adult suite covers entrectinib alone for ROS1-positive advanced NSCLC before any ROS1 inhibitor (00702a), or a separate NTRK gene-fusion solid-tumour indication with no previous NTRK inhibitor and no satisfactory treatment options (00702b). Molecular tests and Irish access criteria must be confirmed. The adult 600 mg schedule does not apply to children.

NCCP 00702 · VERSION 2

Entrectinib monotherapy · adult

The usual adult starting dose is 600 mg by mouth once daily, every day without a planned rest week. Use your prescribed dose. Treatment continues while helping and side effects remain acceptable; there is no fixed number of cycles.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Dizziness, altered thinking or memory

☐ Taste change, nausea, constipation or diarrhoea

☐ Liver injury and low blood counts

☐ Visual disturbance, nerve symptoms and fractures

☐ Heart failure and heart-rhythm changes

☐ Swelling, weight gain and tiredness

☐ Raised uric acid and kidney-test changes

☐ Potential harm in pregnancy

Discuss the exact ROS1 or NTRK indication, alternatives including not proceeding, expected benefit and uncertainty, cognitive/heart/liver risks, fractures, daily capsules, interactions, monitoring and reproductive precautions.

Individual benefit, alternatives and concerns

Agreed checks and next review

Blood count, kidney/liver tests, uric acid and ECG/electrolytes before starting. Repeat ECG/electrolytes after one month, then periodically as indicated; regular blood tests and symptom reviews. Heart-pumping tests depend on symptoms/risk. Scans follow your Consultant’s plan. Confirm who arranges and reviews each check.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.